Life alert

A coordinated path from asking for help to care handover.

REVIEW BASELINE v0.2

Ask for help
Child or parent

→

Coordinate care
Organization + crew

→

Verify handover
Authorized support

THE PRODUCT

Individual care. Clear responsibility.

A US-based subscription service for parents, caregivers and children with disabilities. Family-provided care information supports organizational response and temporary care when appropriate.

What this edition is for

Align the client on the service journey and decisions. Give the designer clear actors, information boundaries, status language and flows. Detailed API contracts remain in the companion technical baseline.

Current position: Django backend foundation with simulated providers. Future Android/iOS app: Flutter. Mobile screens, live integrations and operating arrangements are pending.

01 / PRODUCT INTENT

The service at a glance

A paid care-response service built around each child, with accountable human coordination.

Know the child

Care preferences, communication methods and calming approaches are individual. A diagnosis never defines ability.

Respond together

A linked child or parent can request help. Parent notification runs alongside organizational response and configured EMS escalation.

Complete the handover

Temporary care is considered when appropriate. Follow through until an authorized handover or reviewed disposition is recorded.

CONFIRMED HUMAN DIRECTION

Product choices

Paid new requests require entitlement. Already accepted incidents continue after expiry. Child help has no incident-time parent approval gate.

Technology choices

Django backend first. Flutter for future Android/iOS. Mobile UI waits for client payment and designer screens.

Service boundaries

No confirmed continuous sensor surveillance, universal coverage or guaranteed crew, ambulance, transport or care place. Coverage hours and staffing still need client confirmation.

READ THE LABELS

CONFIRMED

Human-agreed baseline; client validation pending.

PROPOSED

Recommended target or safeguard for review.

PENDING

Client decision or operating dependency still required.

02 / ACTORS + PERMISSIONS

The right information, per role

Access follows household, assignment, region and purpose. Being a contact alone grants no care authority.

Parent / guardian

Manage own household, care profile, contacts, recordings and linked devices.

Cannot manage other families or approve staff actions by default.

Child / linked device

Ask for help independently; view minimal own request status and permitted comfort audio.

No family administration or responder-only instructions.

Regional coordinator

Claim ownership, arrange response, record actions and next steps, reconcile handover.

Authorized regional incidents only; notification is not ownership.

Assigned response crew

Accept offered work; read relevant incident location, care information and instruction audio.

No unrestricted child browsing; access ends with assignment.

Temporary-care staff

Intended access: necessary placement care, intake and authorized release information.

Staff workflows and verified care authority need further delivery.

Configuration administrator

Configure regions, crews, coordinators, facilities and local entitlements with audit.

Admin status alone does not grant child/incident browsing.

Shared boundary: protect sensitive child information

Secure media, revocable devices, object-level controls and an audit trail are required.

03 / FAMILY SETUP

Prepare once. Ask for help simply.

Parent-linked child access is the baseline; child self-registration has not been agreed.

01 Create the family account

Establish guardian access and household ownership.

02 Add the child care profile

Communication, support needs, sensory and calming preferences.

03 Record contacts + authority

Distinguish emergency contacts from authorized care/release recipients.

04 Add purpose-labeled audio

Choose comfort, responder instructions or explicitly both.

05 Link the device + check readiness

Revocable child credential; entitlement and coverage shown separately.

Design for this child

Offer alternatives to reading or speaking. Use simple controls and clear feedback. Account for assistive technology, sensory needs and individual preferences.

Do not infer capabilities from a diagnosis.

Readiness is a set of facts

Identity: device linked?

Payment: entitlement active?

Coverage: area supported?

Care: information current?

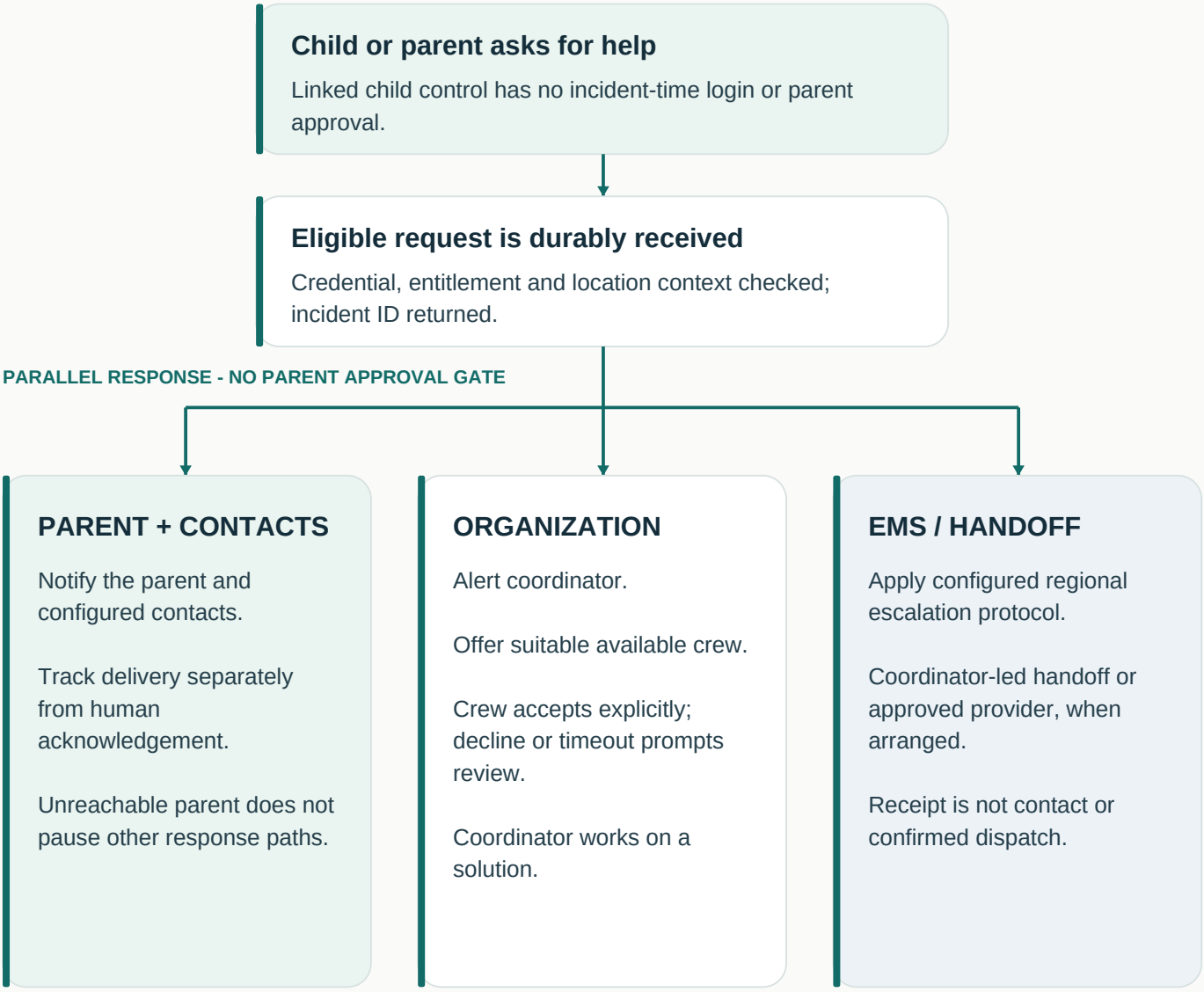
Enrollment is not a guarantee of operational availability.

Designer boundary: no login or parental confirmation at the child's incident-time help action. Device linking happens beforehand.

04 / REQUEST + RESPONSE

One request. Parallel response paths.

Parent acknowledgement never gates organizational response or configured EMS escalation.



Honest feedback at every stage

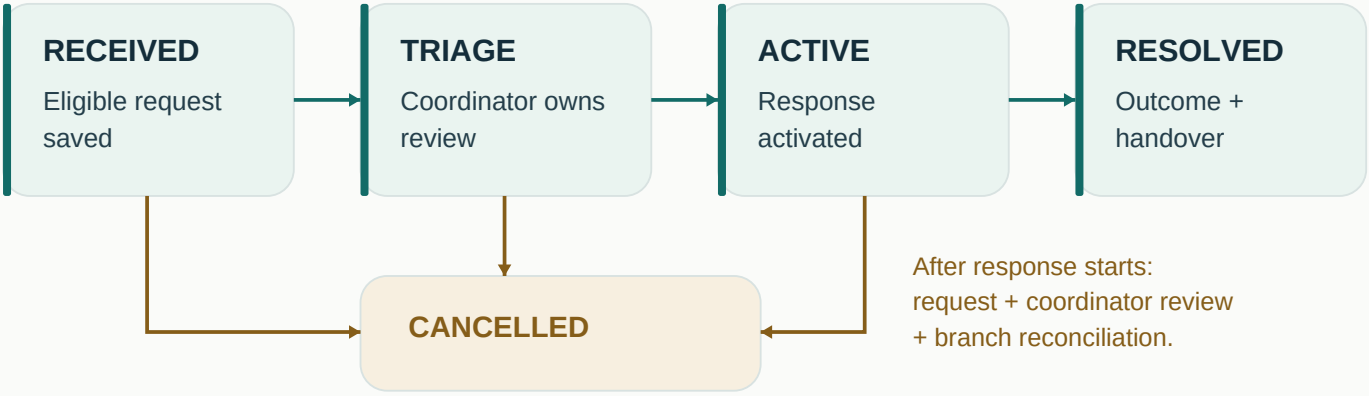
Received, assigned and en-route are different facts. Current development providers simulate outcomes; no real notifications or emergency calls are made.

Location: route from the child/incident, with accuracy and freshness. Saved addresses are labeled fallbacks. Unknown location requires human review.

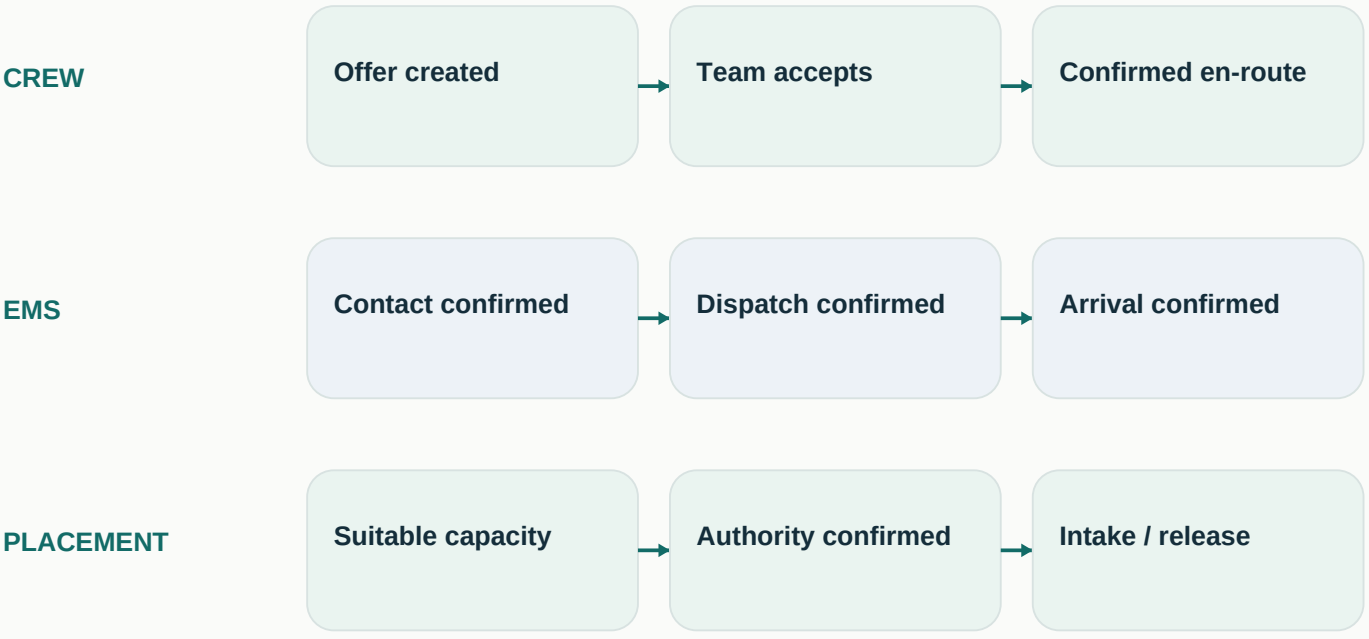
05 / EVIDENCE + STATUS

A status must describe a real event

Keep incident lifecycle separate from notifications, crew response, EMS and temporary-care placement.



INDEPENDENT EVIDENCE TRACKS



Notification delivery is not human acknowledgement

A delivered alert does not mean someone owns the incident. A provider receipt does not confirm EMS dispatch. Assignment acceptance does not mean a crew is already travelling.

06 / ACCOUNTABLE HUMAN RESPONSE

The coordinator works on the solution

Confirmed responsibility: acknowledge ownership, arrange response and follow through to verified resolution.

01 Own the incident

Explicitly acknowledge responsibility; an alert alone does not do this.

02 Arrange appropriate response

Review location and needs; select suitable available support.

03 Record actions + next steps

Keep a clear operational trail and follow-up ownership.

04 Verify handover + disposition

Reconcile crew, EMS and any placement work before closure.

PROPOSED FALLBACK SAFEGUARD

Crew declines / no acknowledgement

Require decline reason or expire the offer using an approved configurable deadline.

Coordinator review + next offer

Alert the responsible coordinator while arranging the next eligible crew.

Backup / supervisor

If primary coordinator remains unresponsive, use the client-approved backup chain. Staffing and timings pending.

Foundation boundary

Manual offers, explicit ownership, progress notes and offer expiry exist. Automatic next-team/backup routing and live delivery are deferred.

07 / TEMPORARY CARE

Temporary care, verified handover

Temporary accommodation is considered when appropriate; purchase does not guarantee a place.

01 Check suitability + capacity

Coordinator reviews child needs and a suitable available facility.

02 Verify authority + consent

Confirm who may authorize placement and under which operating authority.

03 Reserve place + assign staff

Avoid double allocation. Verify staff suitability and responsibility.

04 Arrange transport + intake

Approved transport, relevant care instructions, belongings and intake record.

05 Provide temporary care

Document accountable staff handovers and necessary care events.

06 Verify authorized release

Check recipient identity/authority and record final care handover.

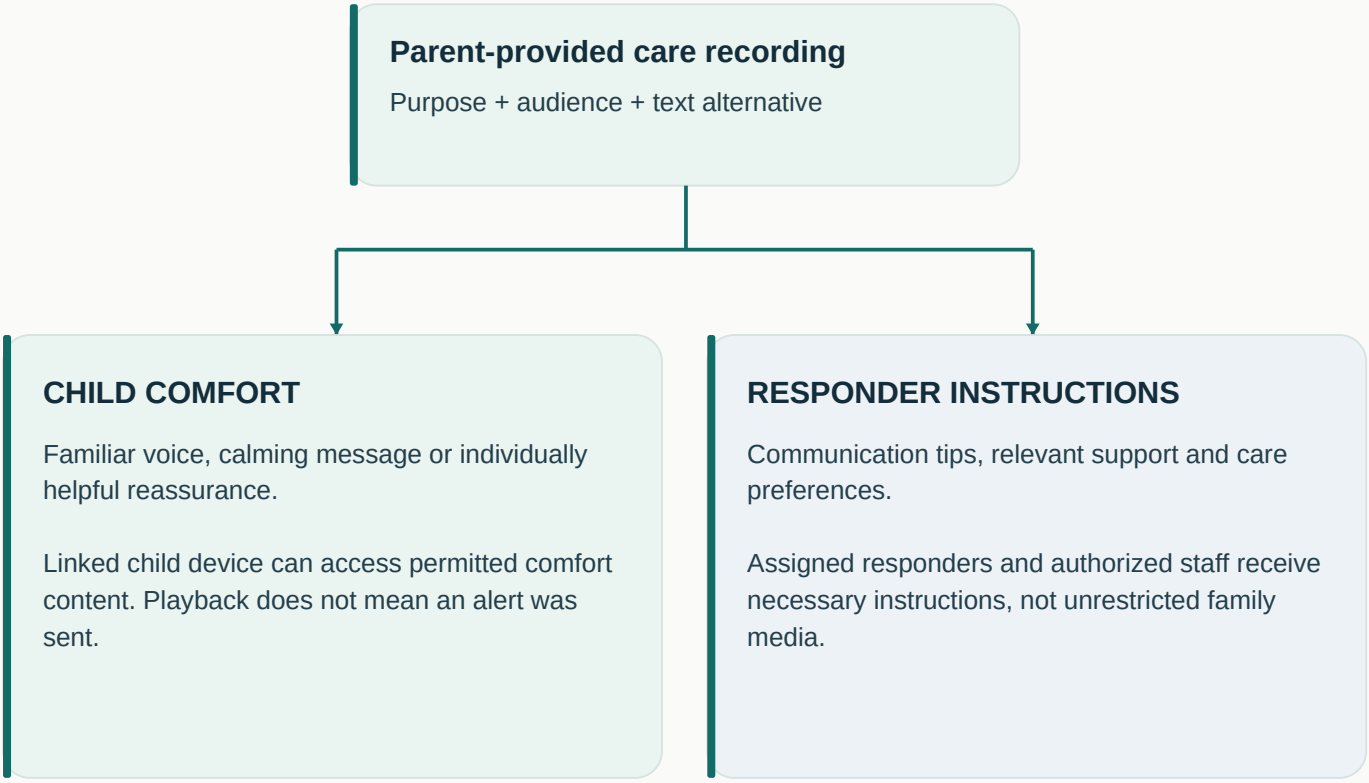
No place available? No authority? Escalate to the coordinator.

Keep responsibility visible and arrange approved alternatives. No temporary care needed? Still record assistance, verified handover and open-response reconciliation before resolving.

08 / CARE CONTENT + INTERACTION

Two recording audiences

A single file may serve both purposes only when its audience access is explicitly chosen.



DESIGN THE EXPERIENCE AROUND EVIDENCE

Accessible help action

Simple, understandable controls. Alternatives to reading/speaking. Individually appropriate feedback. Recover from duplicate taps without creating parallel incidents.

Reassuring status copy

"Your request was received."
"A team has accepted your request."
"Your team is on the way" only after confirmed en-route status.

Privacy boundary: no public audio URLs. Scope access by household, assignment and purpose. Revocation, retention, consent and secure storage require reviewed policies.

09 / ENTITLEMENT + CONTINUITY

Payment controls new paid requests

Human-confirmed baseline: continue already accepted incidents through resolution after expiry.

ALREADY ACCEPTED INCIDENT

Accepted while eligible; same request ID on retry.

Continue the accepted response

Expiry does not abandon care. Preserve the incident and follow through to handover/resolution.

A retry replays the original accepted result; it is not a new paid incident.

Keep these facts separate

Subscription: payment entitlement.

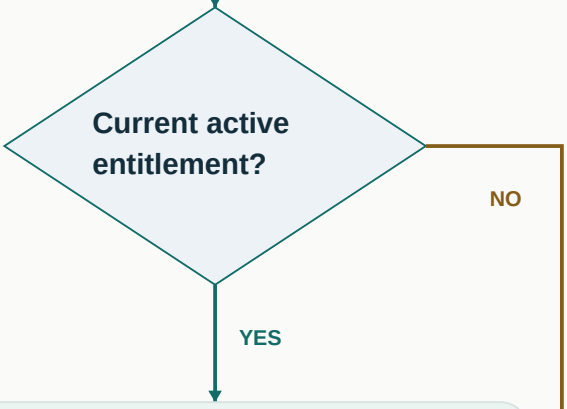
Coverage: operating service area.

Connectivity: delivery capability.

Coverage/network loss does not cancel billing.

NEW PAID REQUEST

New request ID; verify current entitlement.



Check coverage + accept eligibility

Persist eligible request. Missing/stale location requires review; entitlement alone is not a crew or care-place guarantee.

Restrict the new paid request

Explain entitlement status. Renewal is needed for new paid service; the accepted incident still continues.

Ordinary emergency calling remains separate

Available through the supported device calling pathway, independent of paid organization service.

10 / BUILD DIRECTION

Django first. Flutter later.

The current foundation validates backend behavior with synthetic data and simulated integrations.

DEFERRED INTERFACE

Future Flutter app / Android + iOS

Guardian experience and scoped child help experience. Build after client payment and designer screens.

DJANGO + DJANGO REST FRAMEWORK

FAMILY + ACCESS

Profiles, devices, care media

RESPONSE

Entitlements, incidents, ownership, crew states

CARE + AUDIT

Regions, placement, controlled records

BILLING

Local ledger; no payments

NOTIFICATIONS

Simulation; no outbound delivery

EMS / HANDOFF

Simulation/manual record; partner unselected

DELIVERY SEQUENCE

1 Align

Client service model

2 Foundation

Backend + test contracts

3 Design + app

Flutter, when authorized

4 Limited pilot

Only after launch gates

11 / CLIENT REVIEW + LAUNCH GATES

Decisions the client must supply

These choices can be configured; they must be validated before the affected live service launches.

Operating model + authority

Who provides care, coordination, transport and accommodation? What legal/operational authority applies?

Pilot region + coverage

Which US areas and hours? Which ages/support needs are eligible? What service is actually available?

Staffing + escalation ownership

Who owns coordination and response? Qualifications, backup coverage, approved timeouts and handover responsibilities?

Care capacity + authorization

Which facilities, places, staffing and maximum stays? Who can authorize placement and release?

EMS + communications

Which emergency partner/process serves each region? Supported devices/channels? Who confirms contact and dispatch?

Commercial + privacy policies

Pricing, grace/cancellation/refunds; validate continuity. Consent, retention, access, deletion and applicable legal review?

Launch gate: approval plus operational readiness

No live service until staffing, authority, privacy, provider arrangements, recovery and truthful service claims are reviewed. Purchase alone guarantees none of these.

12 / ACCEPTANCE + DESIGN HANDOFF

What a ready experience must prove

Use these scenarios in review. Technical test detail remains in the companion backend guide.

Independent child help	No incident-time login or parental approval. Parent, organization and configured EMS branches are independent.
Duplicates + uncertain location	Retries do not duplicate response. New events and stale/missing location need review. No connectivity means delivery is unconfirmed.
Honest reassurance	Receipt, acknowledgement, assignment, en-route and EMS dispatch are distinct evidence milestones.
Responsible continuity	Expiry blocks new paid requests while accepted care continues. Coordinator owns follow-through.
Care + privacy boundaries	Only authorized audiences access care/audio. Placement and release require suitability, authority and handover.
Failure handling	Nobody accepts? Provider times out? Keep uncertainty and ownership visible; do not invent success.

Review + change control

Record each client change with affected journey/requirement, impact, approval and version. Update this visual PRD and the technical baseline together; formal client sign-off is pending.

Measure separately: receipt-to-acknowledgement, ownership, assignment, en-route, arrival and verified handover. The proposed 5-10 minute mobilization goal needs client validation and a defined milestone; it is not an ETA or guarantee.